



CLINICAL PERFORMANCE LAB
The Cheat Sheet Library

VOL II OF IV

The Universal PE Checklist

Physical exam items that score on every iHuman case

Physical exam is the other ~40%. Most students miss the same handful of items on every case.

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Vol II of IV

The Universal PE Checklist

Physical exam items that score on every iHuman case

Here's the pattern most students miss:

iHuman doesn't just score the PE items that match the chief complaint. It scores a set of universal items that should be performed on virtually any adult clinical encounter — regardless of why the patient came in.

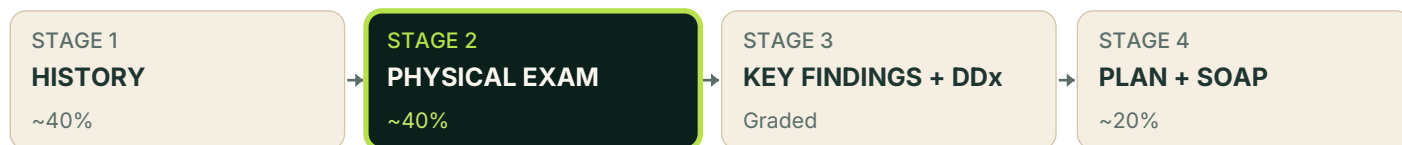
A patient with headaches still needs a cardiac exam. A patient with hypertension still needs a fundoscopic exam. A patient with abdominal pain still needs lung auscultation.

If you're triggering the exam based only on the chief complaint, you'll consistently miss 15–20% of PE points on every case you play.

This sheet covers the universal items — the ones that score on most cases regardless of presentation.

Combine this with a case-specific exam (cardiac complaints → JVP, peripheral pulses; respiratory → percussion, tactile fremitus; etc.) and you'll cover the full PE rubric on any case.

THE IHUMAN CASE FLOW · You are here



Vitals (Always Required)

- ✓ **Cognitive status / level of consciousness** — A&O; × 4
- ✓ Height, weight, BMI
- ✓ Blood pressure — both arms if cardiovascular complaint
- ✓ Heart rate — rate, regularity, strength
- ✓ Respiratory rate — rate, effort
- ✓ Temperature — oral, tympanic, or as available

- ✓ SpO₂ on room air
- ✓ **Critical:** You must view vitals in EHR Current Visit tab for credit. Performing the vital ≠ documenting it.

HEENT — The Universal Items

- ✓ Inspect head — normocephalic, atraumatic
- ✓ **Test visual acuity** (PE item, NOT a test to order)
- ✓ **Examine pupils** — PERRLA
- ✓ **Perform fundoscopic exam** — papilledema check; AV ratio; optic disc
- ✓ Look in ears with otoscope — TMs, canal
- ✓ **Assess hearing** (PE item, NOT a test to order)
- ✓ Inspect nose external/internal
- ✓ Inspect mouth and pharynx
- ✓ Palpate sinuses

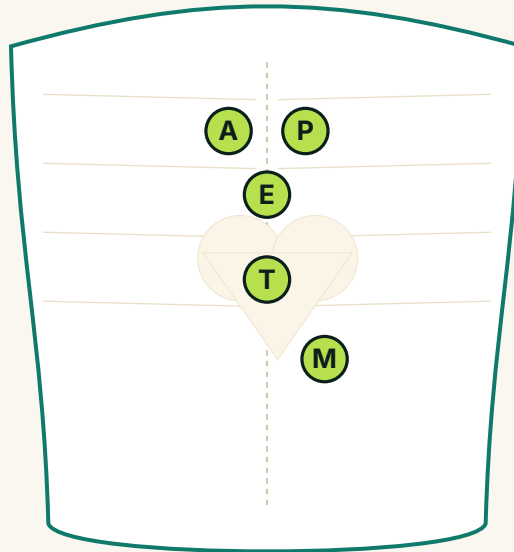
Neck — Always Scored

- ★ Inspect neck — symmetry, masses, JVD
- ★ Palpate cervical lymph nodes
- ★ **Palpate thyroid** — size, nodules, tenderness
- ★ **Auscultate carotid arteries** — bruits
- ★ Evaluate cervical spine ROM (if not contraindicated)

Cardiac — The 5-Point Auscultation

5-POINT CARDIAC AUSCULTATION

Anterior chest, patient supine. Listen with diaphragm at each point.



A Aortic

2nd ICS,
R sternal border

P Pulmonic

2nd ICS,
L sternal border

E Erb's point

3rd ICS,
L sternal border

T Tricuspid

4th ICS,
L sternal border

M Mitral / Apex

5th ICS, MCL
— also PMI

Documenting the Cardiac Exam

Document at minimum: Rate, rhythm, S1/S2, presence/absence of murmurs, gallops, rubs.

Also required on most cases:

Palpate point of maximal impulse (PMI) — laterally displaced PMI = LVH (target organ damage in HTN)

Check peripheral pulses if cardiovascular complaint

Inspect for peripheral edema

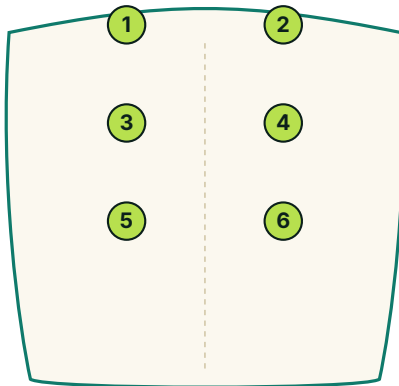
Mnemonic for what to document: Rate, Rhythm, S1/S2, Extra sounds (M-G-R: Murmurs, Gallops, Rubs), Edema.

Lung Field Auscultation

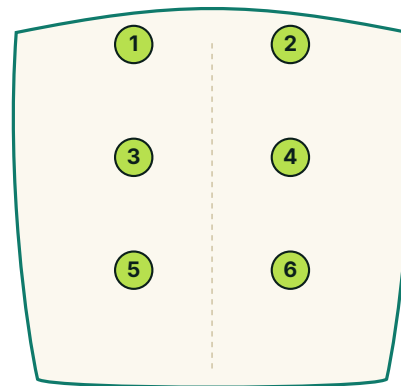
LUNG FIELD AUSCULTATION · 6 anterior + 6 posterior

Compare side to side at each level. Posterior bases first — fluid collects there.

ANTERIOR



POSTERIOR



Documenting Lung Findings

Listen for at each point: Clear vs. crackles, wheezes, rhonchi, decreased breath sounds.

On a respiratory case, also do:

Percussion (resonance vs. dullness)

Tactile fremitus

Inspection — accessory muscle use, retractions

Documentation in the EHR: Lungs CTA bilaterally; no adventitious sounds; respirations even and unlabored. If anything is abnormal, name the lobe and side.

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Abdominal Exam

ABDOMINAL QUADRANTS · Auscultate before palpating

Listen in all four. Document bowel sounds: normoactive / hyperactive / hypoactive / absent.

RUQ Liver, gallbladder, R kidney, head of pancreas, duodenum WATCH FOR • Cholecystitis • Hepatitis	LUQ Spleen, L kidney, stomach, body/tail of pancreas WATCH FOR • Splenomegaly • Gastric ulcer
RLQ Appendix, R ovary, R ureter, cecum, R inguinal ring WATCH FOR • Appendicitis • Ectopic pregnancy	LLQ Sigmoid colon, L ovary, L ureter, L inguinal ring WATCH FOR • Diverticulitis • Ovarian cyst

Quadrants shown from clinician's view (patient's left appears on your right)

The Order Matters: I-A-P-P

Inspect → Auscultate → Percuss → Palpate. Auscultate before palpating — palpation can alter bowel sound activity.

Required steps:

Inspect — contour, scars, visible peristalsis

Auscultate bowel sounds in all 4 quadrants — and interpret (normoactive, hyperactive, hypoactive, absent)

Auscultate for abdominal bruits — renal artery stenosis screen, AAA

Percuss — tympany vs. dullness, liver span

Palpate — light then deep; note tenderness, masses, organomegaly

Check for rebound, guarding, Murphy's sign if right upper quadrant

The trap: Many students auscultate but don't interpret the bowel sounds in the EHR. "Bowel sounds present" loses points; "Normoactive bowel sounds in all four quadrants" earns them.

Neurologic — Standard Mini-Screen

- ✓ Mental status — A&O; × 4, mood, speech
- ✓ Cranial nerves I–XII (especially II, III, IV, VI for vision; V, VII for face; VIII for hearing; IX, X, XII for swallow/speech)
- ✓ Motor — strength testing in upper and lower extremities
- ✓ Sensation — light touch, pain, proprioception
- ✓ Reflexes — biceps, triceps, brachioradialis, patellar, Achilles
- ✓ Coordination — finger-to-nose, heel-to-shin, rapid alternating movements
- ✓ Gait — observe walking, tandem gait, Romberg

PE Items Often Mistaken for Tests

A subtle scoring trap: Some items appear test-like but iHuman classifies them as Physical Exam. Listing them under "Tests Ordered" can trigger a faculty deduction.

These are PE items, not tests:

Visual acuity testing (Snellen)

Hearing assessment (whisper test, finger rub)

Mental status examination (MSE)

Mini-mental state exam (MMSE)

Romberg test

Brudzinski's / Kernig's signs

Dix-Hallpike maneuver

Common PE Traps That Cost Points

⚠ Skipping fundoscopy on cardiovascular cases

Required to rule out hypertensive retinopathy. Missing AV nicking on a HTN case = missing target organ damage = wrong staging.

⚠ Documenting 'bowel sounds present' without interpretation

iHuman expects normoactive / hyperactive / hypoactive / absent. Documenting only 'present' loses partial credit.

⚠ Forgetting PMI palpation on a HTN case

Laterally displaced PMI = LVH. Skipping the palpation means missing one of two target organ damage findings.

⚠ Skipping carotid auscultation on adult cases

Bruits screen is universal on adult cases — required pertinent negative.

⚠ Missing skin turgor on dehydration concerns

Required on peds GE, elderly vomiting/diarrhea, DKA presentations.

⚠ Listing PE items in the Tests Ordered section

Visual acuity, hearing assessment, MSE, Romberg — these are PE items. Listing under Tests triggers a deduction.

Next steps

Next in the library: DDx and Key Findings — how iHuman scores your differential ranking, the must-not-miss framework, and the 2–3 sentence problem statement structure that faculty look for.

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